DIAGNOSTIC IMAGING NETWORK

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Patient Name	: JUAREZ, JUAN	**Patient ID**	: 252634
Date of Birth	: 07/25/1978	**Study Date**	: 08/01/2024 09:26:02
Referring Physician	: CHUIDIAN, CARISSA		
Study Description	: MRI L/SPINE		

CLINICAL HISTORY: Back pain.

TECHNIQUE: Unenhanced MRI of the lumbar spine was imaged in sagittal projection. Axial and coronal images from t12 through s1 were also obtained. The sequences were obtained by established institution protocol.

FINDINGS: There is loss of normal lordotic curvature lumbar spine. The bone marrow signal is within normal limits. The height of the vertebral bodies is preserved. There is no paravertebral soft tissue abnormality.

L1-2: The disc height and signal is within normal limits. There is no disc herniation, central canal stenosis or neural foraminal narrowing. There is no significant facet disease.

L2-3: The disc height and signal is within normal limits. There is no disc herniation, central canal stenosis or neural foraminal narrowing. There is no significant facet disease.

L3-4: The disc height and signal is within normal limits. A 2.5 mm broad based posterior disc herniation resulting in mild central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is no significant neural foraminal stenosis. There is no significant facet disease.

L4-5: The disc height and signal is within normal limits. A 3.0 mm broad based posterior disc herniation resulting in moderate central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is no significant neural foraminal stenosis. There is no significant facet disease.

L5-S1: The disc height and signal is within normal limits. A 2.0 mm broad based posterior disc herniation resulting in mild central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is no significant neural foraminal stenosis. There is no significant facet disease.

IMPRESSION:
1. L3-4: The disc height and signal is within normal limits. A 2.5 mm broad based posterior disc herniation resulting in mild central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is no significant neural foraminal stenosis. There is no significant facet disease.
2. L4-5: The disc height and signal is within normal limits. A 3.0 mm broad based posterior disc herniation resulting in moderate central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is no significant neural foraminal stenosis. There is no significant facet disease.
3. L5-S1: The disc height and signal is within normal limits. A 2.0 mm broad based posterior disc herniation resulting in mild central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is no significant neural foraminal stenosis. There is no significant facet disease.
4. There is loss of normal lordotic curvature lumbar spine. This is a nonspecific finding which may be positional, due to spasm or ligamentous injury. Clinical correlation is recommended.

Thank you for your referral.

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Electronically signed by: KAMRAN KOOCHEK, M.D., DABR, M.S.
Radiologist
T: 08/01/24 #1 KK/am